

MONICA NURSING STUDY CENTER

Clinical Signs

Visual Study Guide

Organized review material

Prepared for study and printing

Signs for Program and NCLEX Success

Meningitis

Figure 64-1 Testing for meningeal irritation. (A) Kernig's sign. (B) Brudzinkski's sign.

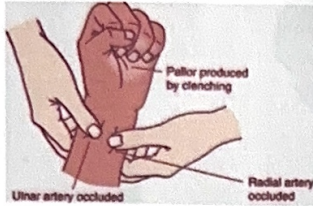
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Brudzinkski's Sign – Flexion of the head causes flexion of both thighs at the hips and knee flexion (neck flexion causes adduction and flexion movements of the lower extremities)

Kernig's Sign – Flexion of the thigh and knee to right angles, and when the limbs are extended. It causes spasm of the hamstring with pain if positive (inability to extend the leg when the thigh is flexed interiorly at the hip).

Nuchal Rigidity – Refers to a stiffness that prevents bending of the neck and limits movement of the neck.

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ABG/Blood Flow Patency**ALLENS Test**

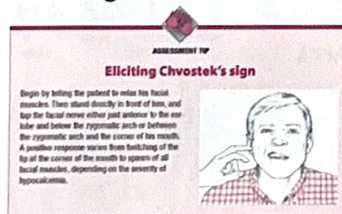
Apply pressure over the patient's ulnar and radial arteries simultaneously then ask the client to open and close their hand. The hand should blanch; release the pressure from ulnar artery while still applying pressure to the radial artery to assess for patency. If pinkness fails to return in 6 seconds, artery is not patent and should not be used for an ABG stick.

Hypocalcemia Signs (Tetany)

Trousseau's Sign – Carpal spasm is elicited by pressure on brachial artery with a blood pressure cuff for 3 minutes

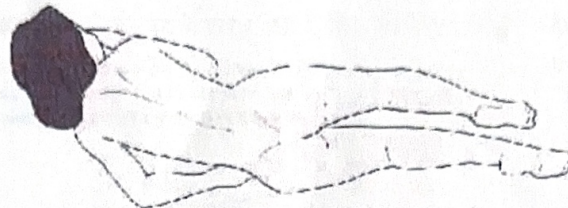


CHVOSTEKS Sign – Tapping on face anterior to the ear, just below zygomatic bone causes twitching of facial muscle's

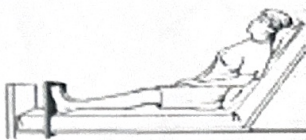


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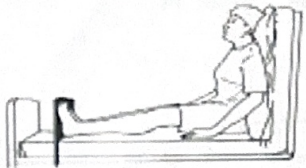
Positioning



Sims' (left-lateral) Position



Semi Fowlers

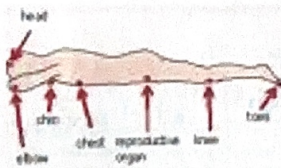


High Fowlers

MF2184, MF2186 (A)
MF2186 (B), MF2187



Reverse Trendelenburg



Prone

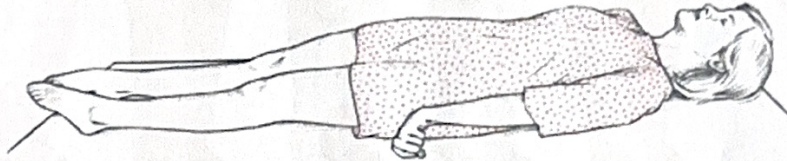


Side-Lying

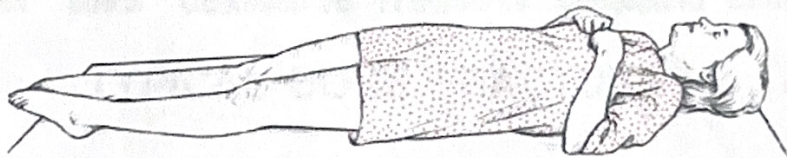
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Decerebrate/Decorticate Posturing**Comparing decerebrate and decorticate postures**

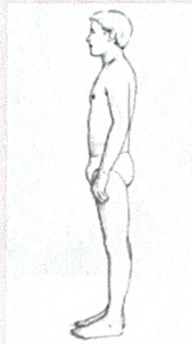
Decerebrate posture results from damage to the upper brain stem. In this posture, the arms are adducted and extended, with the wrists pronated and the fingers flexed. The legs are stiffly extended, with plantar flexion of the feet.



Decorticate posture results from damage to one or both corticospinal tracts. In this posture, the arms are adducted and the elbows are flexed, with the wrists and fingers flexed on the

**Romberg's Sign****ASSESSMENT TIP****Assessing Romberg's sign**

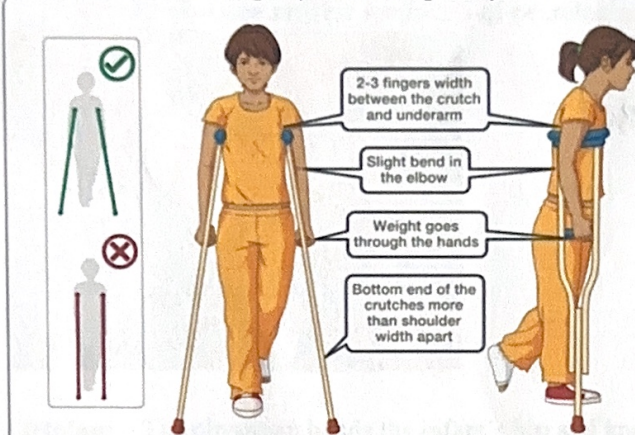
Observe the patient's balance as he stands with his eyes open, feet together, and arms at his sides. Then ask him to close his eyes. Hold your arms out on either side of him to protect him if he sways. If he falls to one side, the result of the Romberg's test is positive.



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Weight Bearing Terms

Stairs/crutch teaching- Up with the good (unaffected leg) and down with the bad (affected leg).



Partial Weight Bearing – With little weight on the weak/injured foot

Non-Weight Bearing – The weak foot/ankle is off the floor. Crutches are held in front and to the side of the body “swings” through

Reproductive Signs

Chadwick’s Sign - “Probable” sign of 4 weeks into pregnancy due to increase vascularity of the cervix, vagina, and vulva a dark blue/violet color

Goodell’s Sign – “Probable” sign of pregnancy in which the cervix softens 4-6 weeks into pregnancy

Hegar’s Sign – “Probable” sign of pregnancy that occurs 6 weeks in which uterine wall softens

Nagele’s Rule →

Naegle’s Rule – Add 7 days to the first day of LMP, subtract 3 months and add one year.

Positive Signs of Pregnancy/Diagnostic

Ultrasound/radiological outline of fetus

Fetal heart rate Doppler at 10-12 weeks

Active fetal movement palpable by examiner

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Pediatric Signs

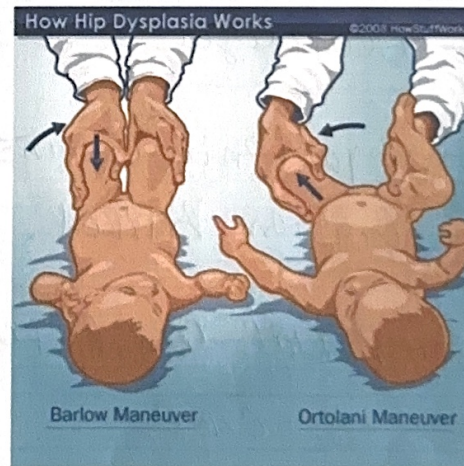
Babinski Reflex – Presence of absence (**not** positive or negative) of Babinski reflex is:

- Expected to be **present** in babies up to about 12 months old
- Its **absence suggest symptoms** of neurological concerns



Ortolani – The physician bends the infant's hip and knees to 90 degrees. Once in this position, the doctor **abducts**, or moved the infant's legs apart, into the frog-leg position. If the hip is dislocated, the femoral head slides into the hip socket during this movement. There is a palpable and audible clunk as the head slops back into the socket. Each hip should be tested individually. Hearing or feeling the clunk is called a **positive Ortolani maneuver**

Barlow - The hip will gently be pushed posteriorly – toward the back – along the line of the femur shaft. In this case, a **positive Barlow test** occurs if the physician feels the femoral head slip out of the acetabulum as the hip dislocates out of the socket

Rash Signs

Scarlet Fever – Pastia's sign is a rash typically seen in skin folds that will blanch with pressure except in the area of deep creases.

- Tongue **initially** covered with white furry tongue (white strawberry tongue) then sloughs off on the 4th or 5th day (swollen red strawberry tongue) remains.
- The pharynx is understandably edematous and beefy red

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Rubeola (measles) – Koplick spots are associated with Rubeola (measles) which are small red spots with a bluish white center and a red base located on the buccal mucosa.



German Measles – Petechial red, pinpoint spots occurring on the soft palate are characteristic of rubella

Notes
